

GUIDELINES FOR FOREIGN BODY INGESTION IN THE CHILDREN'S EMERGENCY

1. MANAGEMENT

EMERGENT endoscopic removal

- STAT admission to PICU for endoscopic removal of FB ideally within 2 hours of presentation
- Activate GI-on-call immediately from CE for urgent endoscopy
- Activate PICU-on-call for admission

URGENT endoscopic removal (within 24 hours of presentation)

- Admit to ward and repeat X-ray upon admission or ≥ 4 hours from last X-ray; FB may have spontaneously passed distally
- If extended wait for bed in CE, to repeat X-ray ≥ 4 hours from last X-ray → if FB has spontaneously passed distally, no more indication for EMERGENT endoscopic removal
- Discharge from CE with outpatient follow-up in Pediatric clinic the following morning (if caregivers refuse admission)

1.1 **DISPOSITION**

- Disposition can be amended at discretion of CE senior/GI-on-call if FB progresses on serial X-ray (e.g., esophageal FB passes into stomach on repeat X-ray, patient may be discharged instead of being admitted)

1.2 **SURGICAL REFERRAL**

- Activate PAS STAT if child presents with complications (e.g., perforated viscus with mediastinitis, peritonitis, perforated bowel / acute abdomen etc)

2. ACUITY OF ENDOSCOPIC REMOVAL

TYPE OF FB	SITE	SYMPTOMS* (YES/NO)	DISPOSITION
Button battery (BB)	Esophagus	Yes / No	Emergent
	Stomach	Yes	Urgent
	Stomach	No With risk factors of: ▪ Age < 5-years, AND ▪ BB > 2cm in diameter	Urgent
	Stomach	No	Discharge TCU GI in 48 to 72H
Sharp objects ▫ Unlocked safety pin ▫ Nails, tacks, hijab pins	Esophagus / Stomach	Yes / No	Emergent
Multiple magnets	Esophagus / Stomach	Yes / No	Emergent
	Small bowel	Yes / No	Activate PAS
Single magnet	Esophagus	Yes	Emergent
	Esophagus	No	Urgent
	Stomach		Discharge TCU GI in 1 – 2 weeks Advise parent to avoid FURTHER magnet ingestion
Coin (Regardless of diameter)	Esophagus	Yes	Emergent
	Esophagus	No	Urgent
	Stomach		Discharge TCU GI in 1 – 2 weeks
Long blunt object (> 6cm in length)	Esophagus / Stomach	Yes	Emergent
		No	Urgent
Food bolus	Esophagus	Yes (& unable to swallow)	Emergent
	Esophagus	No	Urgent
All other FBs not otherwise categorized e.g., blunt objects < 6cm	Esophagus	No	Urgent
	Stomach	Yes	Urgent
	Stomach	No	Discharge TCU GI in 1 week

*Symptoms attributable to the foreign body ingestion

3. REFERENCES

1. Kramer RE, Lerner DG, Lin T, Manfredi M, Shah M, Stephen TC, Gibbons TE, Pall H, Sahn B, McOmber M, Zacur G, Friedlander J, Quiros AJ, Fishman DS, Mamula P; North American Society for Pediatric Gastroenterology, Hepatology, and Nutrition Endoscopy Committee. Management of ingested foreign bodies in children: a clinical report of the NASPGHAN Endoscopy Committee. *J Pediatr Gastroenterol Nutr.* 2015 Apr;60(4):562-74. doi: 10.1097/MPG.0000000000000729. PMID: 25611037.
2. Mubarak A, Benninga MA, Broekaert I, Dolinsek J, Homan M, Mas E, Miele E, Pienar C, Thapar N, Thomson M, Tzivnikos C, de Ridder L. Diagnosis, Management, and Prevention of Button Battery Ingestion in Childhood: A European Society for Paediatric Gastroenterology Hepatology and Nutrition Position Paper. *J Pediatr Gastroenterol Nutr.* 2021 Jul 1;73(1):129-136. doi: 10.1097/MPG.0000000000003048. PMID: 33555169.